

AI Claim Adjudication Note

Request 941b06e0 | 110202496617-1.pdf | Generated 2026-07-04 12:57:35 UTC

INVESTIGATE

Do not auto-approve

Blocking problems were found. Resolve the high-severity findings below first.

Issues raised: 22 high / 10 medium / 2 low

Claim no.	—	Patient	MRS. HARSHITA SHARMA W/O MR.SHIVAM TIWARI (28 Y 1 M 11 D / Female)
Diagnosis	Acute Febrile Illness // Viral Hepatitis - HAV(+)	Treatment	Conservative management with appropriate medical treatment including IV fluids and supportive care, with regular monitoring of vitals, blood glucose, input-output, and other relevant parameters.
Hospital	M.P.BIRLA HOSPITAL SATNA	Insurer / Policy	ICICI LOMBARD / —
Admitted -> Discharged	2/06/2026 7:49 PM -> 8/06/2026 6:01 PM (6 daysd)	Treating doctor	DR. ASHISH MISHRA

Financial snapshot

Total billed Rs. 74,254	Recommended payable Rs. 73,919	Non-payable Rs. 335	Undetermined items 0
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Domain scorecards OK CHECK RISK

Document Checker	
Overall verdict: INCOMPLETE	RISK
Mandatory docs present: 5/16 (31%)	RISK
Missing critical docs: 1 (Identity Proof - Aadhar)	RISK
Claim type: Unable to detect	OK
Recommended action: Request Missing Documents	RISK
Document check failed: the claim file is severely incomplete. 5 of 16 mandatory documents are present (31.2% complete, status critical). Critical documents missing: Identity Proof - Aadhar. The claim type (cashless vs reimbursement) could not be determined from the documents. Recommended action: REQUEST_MISSING_DOCUMENTS.	

Administrative Review	
Overall verdict: WARN	CHECK
Identity: PASS	OK
Insured: SKIPPED	CHECK
Policy Dates: SKIPPED	CHECK
Period Compliance: PASS	OK
Blocking issues: 0	OK
Administrative admissibility cleared with advisory flags for human review. Checks: identity=PASS, insured=SKIPPED, policy_dates=SKIPPED, period_compliance=PASS. Advisory notes: insured: Missing patient name or insured name — cannot verify insured.; policy_dates: Cannot check policy period — missing/unparseable: policy_start_date, policy_end_date..	

Medical Admissibility	
Overall verdict: INVALID	RISK
Treatment validity score: 0.53	CHECK
Charges valid: 114/233	OK
Charges invalid: 19	RISK
Charges to review: 100	CHECK
Major conflicts: 4	RISK
Treatment process correct: No / unconfirmed	CHECK
PED validation: INSUFFICIENT	CHECK
Medical admissibility failed: one or more charges are not clinically admissible. Treatment validity score 0.53; recommended action REJECT. 233 charge(s): valid=114, partial=18, invalid=19, review=82. Treatment process could not be confirmed: Treatment process is clinically inconsistent due to unexplained medications, duplicate charges, LOS mismatch, and lack of lab/investigation support. Major conflicts: Length of stay mismatch between billing and discharge summary: 6 vs 4; Treating doctor mismatch between billing and discharge summary: DR. ASHISH MISHRA vs Ashish Mishra; 170 line items appear duplicated by description and amount; Negative billing amounts are not admissible. PED validation: INSUFFICIENT / MANUAL_REVIEW across 1 pre-existing disease(s).	

Financial Adjudication	
Overall verdict: INVESTIGATION_REQUIRED	CHECK
Total billed: Rs. 74,254.06	N/A
Admissible / payable: Rs. 73,919.26	OK
Non-payable: Rs. 334.80	CHECK
Undetermined line items: 0	OK
Bill reconciles: No	RISK
Financial adjudication is inconclusive and requires investigation. Of 74254.06 billed across 198 line item(s): payable 73919.26 (195), non-payable 334.80 (3), undetermined 0.00 (0). Recommended action INVESTIGATE; admissible amount 73919.26. The bill does not reconcile (line items 74254.06 vs summary 70702.62). Billing type assessed as Medical Management. Figures derived via the consolidated path.	

Findings & required actions

Severity	Area	Finding	Suggested action
HIGH	Document Checker	Critical mandatory document missing: Identity Proof - Aadhar.	Request Identity Proof - Aadhar from the hospital / claimant before adjudication.

Severity	Area	Finding	Suggested action
HIGH	Document Checker	Claim file is severely incomplete: 11 mandatory documents missing.	—
HIGH	Medical Admissibility	Charge #0 is INVALID: Rejected by deterministic validation checks [charge[0]]	—
HIGH	Medical Admissibility	Charge #1 is INVALID: Rejected by deterministic validation checks [charge[1]]	—
HIGH	Medical Admissibility	Charge #2 is INVALID: Rejected by deterministic validation checks [charge[2]]	—
HIGH	Medical Admissibility	Charge #3 is INVALID: Rejected by deterministic validation checks [charge[3]]	—
HIGH	Medical Admissibility	Charge #4 is INVALID: Rejected by deterministic validation checks [charge[4]]	—
HIGH	Medical Admissibility	Charge #5 is INVALID: Rejected by deterministic validation checks [charge[5]]	—
HIGH	Medical Admissibility	Charge #6 is INVALID: Rejected by deterministic validation checks [charge[6]]	—
HIGH	Medical Admissibility	Charge #7 is INVALID: Rejected by deterministic validation checks [charge[7]]	—
HIGH	Medical Admissibility	Charge #8 is INVALID: Rejected by deterministic validation checks [charge[8]]	—
HIGH	Medical Admissibility	Charge #9 is INVALID: Rejected by deterministic validation checks [charge[9]]	—
HIGH	Medical Admissibility	Charge #194 is INVALID: Rejected by deterministic validation checks [charge[194]]	—
HIGH	Medical Admissibility	Charge #195 is INVALID: Rejected by deterministic validation checks [charge[195]]	—
HIGH	Medical Admissibility	Charge #196 is INVALID: Rejected by deterministic validation checks [charge[196]]	—
HIGH	Medical Admissibility	Charge #197 is INVALID: Rejected by deterministic validation checks [charge[197]]	—
HIGH	Medical Admissibility	Charge #198 is INVALID: Rejected by deterministic validation checks [charge[198]]	—
HIGH	Medical Admissibility	Charge #199 is INVALID: Rejected by deterministic validation checks [charge[199]]	—
HIGH	Medical Admissibility	Charge #200 is INVALID: Rejected by deterministic validation checks [charge[200]]	—
HIGH	Medical Admissibility	Charge #230 is INVALID: Pulmonology visit not clinically justified for HAV; no respiratory involvement documented. [charge[230]]	—
HIGH	Medical Admissibility	Charge #231 is INVALID: Duplicate pulmonology visit; no respiratory indication for HAV; duplicate charge flagged. [charge[231]]	—
HIGH	Financial Adjudication	Bill does not reconcile: line items sum to 74254.06 vs summary 70702.62 (difference +3551.44).	—
MEDIUM	Document Checker	10 mandatory document(s) missing: Claim Form, Authorization Letter, Pre Authorization Request Note, Investigation Reports, Payment Receipts, Indoor Case Papers, Implants Invoice, Implants Label or Sticker, Prescription Papers, Others.	—
MEDIUM	Document Checker	Could not determine whether this is a cashless or reimbursement claim.	—
MEDIUM	Medical Admissibility	82 charge(s) require manual review.	—
MEDIUM	Medical Admissibility	Length of stay mismatch between billing and discharge summary: 6 vs 4	—
MEDIUM	Medical Admissibility	Treating doctor mismatch between billing and discharge summary: DR. ASHISH MISHRA vs Ashish Mishra	—
MEDIUM	Medical Admissibility	170 line items appear duplicated by description and amount	—
MEDIUM	Medical Admissibility	Negative billing amounts are not admissible	—

Severity	Area	Finding	Suggested action
MEDIUM	Medical Admissibility	Treatment process not confirmed: Treatment process is clinically inconsistent due to unexplained medications, duplicate charges, LOS mismatch, and lack of lab/investigation support.	—
MEDIUM	Medical Admissibility	PED validation requires manual review (ambiguous or undeclared evidence).	—
MEDIUM	Financial Adjudication	3 line item(s) were flagged for review during consolidation.	—
LOW	Administrative Review	insured: Missing patient name or insured name — cannot verify insured.	—
LOW	Administrative Review	policy_dates: Cannot check policy period — missing/unparseable: policy_start_date, policy_end_date.	—

Adjudicator summary: The claim cannot be auto-approved and must be investigated. Blocking agents: Document Checker, Medical Admissibility. Agents needing review: Administrative Review, Financial Adjudication. 34 issue(s) raised (22 high, 10 medium, 2 low) across 4 agent(s).

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